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Pain in Emergency Medicine: The Importance of Rapid Analgesia in the Emergency Department

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Abstract

Background. Acute pain is one of the most common and clinically important problems in emergency medicine. Despite available pharmacological and non-pharmacological treatments, pain is frequently underestimated, undertreated, delayed, or insufficiently reassessed. This persistent phenomenon, known as oligoanalgesia, may adversely affect patient comfort, satisfaction, diagnostic cooperation, and the quality of emergency care.

Aim. This narrative review evaluates current approaches to rapid analgesia in the emergency department, focusing on early pain assessment, reassessment, multimodal analgesia, opioid-sparing strategies, and organizational barriers to effective pain management.

Materials and Methods. A narrative review of full-text scientific literature published between 2017 and 2026 was conducted. The literature search focused on acute pain, emergency department, rapid analgesia, oligoanalgesia, pain assessment, multimodal analgesia, and emergency medicine.

Results. Evidence indicates that inadequate assessment, delayed treatment, and insufficient reassessment remain major barriers to effective pain management. Structured assessment, early analgesic intervention, reassessment, multimodal strategies, and appropriate use of non-opioid and opioid-sparing approaches may improve pain management. Individualized strategies are particularly important for children, older adults, pregnant patients, and patients with complex or life-limiting conditions.

Conclusions. Rapid analgesia should be an integral component of emergency department care. Systematic assessment, timely treatment, reassessment, multimodal analgesia, staff education, and structured clinical pathways may reduce oligoanalgesia and improve patient-centered outcomes.

Keywords: acute pain, emergency department, rapid analgesia, oligoanalgesia, pain assessment, multimodal analgesia, emergency medicine

1. Introduction

Pain is one of the most common reasons for seeking emergency medical care and is a major therapeutic challenge in the emergency department. Acute pain may occur in patients with trauma, fractures, dislocations, renal colic, abdominal pain, headache, burns, sickle cell pain episodes, cancer-related pain, musculoskeletal disorders, painful procedures, and many other emergency conditions. Because pain is both a symptom and a source of suffering, emergency clinicians must evaluate it not only as a diagnostic signal but also as an independent therapeutic target.

The term oligoanalgesia is used to describe insufficient or delayed pain treatment in patients who require analgesia. In emergency medicine, oligoanalgesia may result from poor pain documentation, lack of reassessment, diagnostic uncertainty, fear of opioid-related adverse effects, crowding, workload, communication failures, and absence of structured analgesic pathways. The problem is clinically important because untreated or undertreated pain may increase distress, reduce cooperation, worsen the patient experience, and undermine the quality of emergency care.

Contemporary emergency pain management is based on early recognition, appropriate analgesic selection, timely administration, reassessment, and adjustment of treatment. Modern approaches emphasize multimodal analgesia, which combines medications and non-pharmacological interventions with different mechanisms of action. This strategy may improve analgesic effectiveness while reducing unnecessary reliance on opioids. Rapid analgesia does not mean indiscriminate drug administration, but rather timely, individualized, safe, and reassessed treatment adapted to the patient's pain intensity, diagnosis, comorbidities, age, preferences, and risk profile.

The aim of this review is to summarize the importance of rapid analgesia in the emergency department and to discuss current evidence on pain assessment, oligoanalgesia, multimodal analgesia, non-opioid strategies, opioid-sparing approaches, special populations, and organizational solutions designed to improve pain care.

2. Research materials and methods

2.1. Participants

This study is a narrative review and did not involve direct participation of patients, healthy volunteers, or healthcare professionals. No clinical intervention was performed, no individual

medical records were analyzed, and no personal data were collected. Therefore, institutional review board approval and informed consent were not required.

2.2. Search strategy and selection of literature

The literature search focused on full-text publications from 2017 to 2026 related to acute pain and emergency medicine. The following search terms and combinations were used: “acute pain”, “emergency department”, “rapid analgesia”, “oligoanalgesia”, “pain assessment”, “multimodal analgesia”, and “emergency medicine”. Priority was given to publications with full-text availability and identifiable DOI numbers.

The included literature consisted of clinical reviews, narrative reviews, systematic reviews, meta-analyses, randomized clinical trials, observational studies, quality improvement studies, consensus documents, guidelines, and protocols relevant to pain assessment and analgesia in emergency care. Publications were included if they addressed acute pain assessment, reassessment, pharmacological analgesia, non-pharmacological analgesia, multimodal treatment, nurse-initiated analgesia, trauma pain, pediatric emergency pain, special populations, opioid-sparing strategies, ketamine, methoxyflurane, intranasal analgesia, or organizational barriers in emergency departments. Publications were excluded if they were unrelated to acute pain in emergency care, focused primarily on chronic pain without an emergency department context, or lacked reliable bibliographic data.

2.3. Data collection and analysis

The selected articles were analyzed narratively. The extracted themes included the clinical importance of acute pain in emergency medicine, pain assessment and reassessment, oligoanalgesia, time to analgesia, pharmacological options, non-opioid strategies, opioid use, ketamine, inhaled and intranasal analgesia, trauma pain, pediatric pain, special populations, and organizational barriers. Because this was not a systematic review or meta-analysis, no pooled statistical analysis was performed.

2.3.1. Statistical Software

No statistical software was used because this study did not include original quantitative analysis or meta-analysis.

2.3.2. AI

Artificial intelligence was used only as an auxiliary tool to organize source material, support linguistic editing, and improve the clarity and structure of the manuscript. The selection of

scientific arguments, interpretation of literature, and final responsibility for the content remain with the author.

2.3.3. Statistical Methods

No original statistical methods were applied. The evidence was synthesized descriptively according to clinically relevant themes in emergency pain management.

3. Research results

3.1. Acute pain as a major clinical problem in emergency medicine

Acute pain is a central clinical problem in emergency medicine because it is common, distressing, diagnostically relevant, and directly related to patient experience and quality of care [1,3,5,7].

The reviewed literature shows that pain in the emergency department should be considered both a symptom requiring diagnostic evaluation and a condition requiring active treatment [1,2,3,5].

Prospective and review-based studies describe acute pain as a frequent component of emergency department presentations and identify persistent gaps between pain recognition and adequate analgesic treatment [7,8,9,10].

Emergency pain care is complex because clinicians must manage pain while also addressing diagnostic uncertainty, patient flow, crowding, medication safety, and competing clinical priorities [3,8,19,20].

The persistence of oligoanalgesia across different clinical contexts indicates that inadequate emergency pain treatment is not limited to a single diagnosis, drug class, hospital, or patient group [3,7,19,23].

The reviewed literature also shows that pain management is clinically relevant in both general emergency presentations and specific groups such as trauma patients, children, patients with severe pain, and patients at the end of life [22,23,31,32].

3.2. Pain assessment and reassessment

Pain assessment is the necessary first step in emergency analgesic care because treatment decisions depend on recognizing pain intensity, clinical context, patient characteristics, and response to intervention [1,9,10,11].

The EUSEM assessment guideline emphasizes structured evaluation of acute pain in emergency settings and supports systematic approaches to pain recognition [1].

Patient-reported pain scores remain important in emergency practice because they provide a direct measure of the patient's subjective pain experience [1,10,11,12]. However, the reviewed literature indicates that pain scores should not be interpreted in isolation because analgesic need may also depend on function, distress, diagnosis, patient preference, and safety considerations [10,13,14,15].

Studies focused on pain scoring and analgesic requirement show that unidimensional scales may be practical but may not fully capture the multidimensional nature of emergency department pain [11,12,13].

The international work on defining the need for analgesia reflects growing recognition that pain intensity alone is not always sufficient to determine treatment priority in the emergency department [14,15].

Reassessment after analgesia is essential because initial treatment may be insufficient, delayed, excessive, or associated with adverse effects [1,2,3,16].

Educational and organizational interventions may improve pain assessment and documentation, but improvement in assessment alone does not guarantee timely and effective analgesia [16,17,19].

3.3. Oligoanalgesia and delayed analgesia

Oligoanalgesia remains a persistent problem in emergency medicine despite the availability of multiple analgesic drugs and evidence-based treatment strategies [3,5,7,19]. The reviewed studies identify underassessment, lack of documentation, delayed prescribing, delayed administration, insufficient reassessment, and absence of standardized protocols as contributors to inadequate pain relief [9,16,19,20].

A patient-centered perspective is important because patients may experience pain care as inadequate even when clinicians believe that pain has been assessed or treated [9,10,11]. Quality improvement literature suggests that pain management failures may occur at multiple points of the emergency department pathway, including triage, clinical assessment, prescribing, medication delivery, and reassessment [17,19,20].

Nurse-initiated analgesia has been investigated as a strategy to reduce delay because it can allow selected patients to receive pain treatment before complete physician evaluation [17,18]. The effectiveness of nurse-initiated analgesia depends on patient factors, nurse-related factors, workload, protocol design, medication availability, and institutional support [17,18,19]. The evidence indicates that rapid analgesia requires not only appropriate drug choice but also a workflow that permits early assessment, early treatment, and repeated evaluation [1,2,17,20].

The main clinical areas involved in rapid analgesia include early pain assessment, interpretation of analgesic need, timely treatment, multimodal analgesia, reassessment, and organizational quality improvement, as summarized in Table 1 [1–5,9–20,22,23,30,36–43].

Table 1. Clinical framework for rapid analgesia in the emergency department

Clinical area	Main problem in the ED	Practical approach	Why it matters
1. Initial pain assessment	Pain may be underrecognized or poorly documented.	Assess pain at triage using a standardized scale plus clinical context, distress, and functional limitation.	Early recognition helps identify patients who need prompt analgesia.
2. Interpretation of pain intensity	A numeric score alone may not reflect analgesic need.	Interpret pain scores together with diagnosis, patient preference, functional impairment, emotional distress, and clinical risk.	Prevents both undertreatment and overly rigid treatment decisions.
3. Early analgesic decision	Analgesia may be delayed while waiting for physician review or diagnostics.	For moderate or severe pain, make an early analgesic decision, including protocolized or triage-based analgesia when appropriate.	Reduces unnecessary suffering and supports patient-centered care.
4. Non-pharmacological measures	Pain relief is sometimes reduced to medication alone.	Use immobilization, splinting, positioning, ice, reassurance, and procedural preparation as supportive measures.	These methods complement medication and reduce discomfort and anxiety.
5. Pharmacological analgesia	A single-drug approach may be insufficient or unsafe.	Individualize treatment using acetaminophen, NSAIDs, opioids,	Different pain mechanisms and patient risks require flexible treatment.

		ketamine, regional techniques, inhaled analgesia, or intranasal routes as appropriate.	
6. Multimodal analgesia	Reliance on one drug class may increase adverse effects or provide incomplete relief.	Combine pharmacological and non-pharmacological methods according to pain severity, contraindications, and expected clinical course.	Improves analgesic effect and can reduce opioid exposure.
7. Opioid-sparing strategies	Severe pain may require opioids, but unnecessary opioid exposure remains a concern.	Consider non-opioid analgesics, ketamine, methoxyflurane, intranasal analgesia, and regional techniques when appropriate.	Supports effective pain control while limiting opioid-related risks.
8. Trauma pain	Trauma patients often need stabilization, imaging, and procedures at the same time as analgesia.	Assess and treat trauma pain early during the diagnostic process instead of postponing pain relief.	Analgesia can be integrated with safe trauma care and serial reassessment.
9. Special populations	Children, older adults, pregnant patients, sickle cell patients, cancer patients, and end-of-life patients have distinct needs and risks.	Adapt analgesia to age, communication ability, comorbidities, organ function, pregnancy/lactation status, disease-specific pain syndromes, and goals of care.	Rapid analgesia must remain individualized in vulnerable groups.
10. Reassessment after analgesia	Initial treatment may be ineffective, incomplete,	Reassess pain, function, distress, sedation, respiratory status, and	Allows rescue analgesia, dose adjustment, and safer ongoing care.

	excessive, or associated with adverse effects.	adverse effects after treatment.	
11. Organizational quality improvement	Oligoanalgesia often reflects system-level barriers.	Use staff education, triage protocols, nurse-initiated analgesia, documentation prompts, audit, feedback, and reassessment standards.	System-level improvement increases the timeliness and reliability of analgesia.

3.4. Multimodal analgesia and non-opioid strategies

3.5. Ketamine, methoxyflurane, intranasal analgesia, and other rapid analgesic options

Low-dose ketamine has been studied in emergency department acute pain as an opioid-sparing alternative or adjunct in selected patients [30,41,42].

Systematic review evidence suggests that ketamine can reduce acute pain in emergency department patients, although transient adverse effects and appropriate patient selection remain important clinical considerations [30,41].

Randomized clinical trial evidence comparing low-dose ketamine and morphine supports ketamine as a relevant emergency analgesic option, but the choice between ketamine and opioids should be individualized [30,41].

Nebulized ketamine has been described as a non-invasive analgesic route, but the available evidence is less mature than for established intravenous, oral, intranasal, or inhaled strategies [42,43].

Intranasal analgesia may be useful when intravenous access is delayed, difficult, unnecessary, or undesirable, particularly in settings where rapid treatment is needed and patient selection is appropriate [32,40,43].

Methoxyflurane has been reviewed as an inhaled analgesic option for acute pain and trauma-related pain, with potential usefulness when rapid, supervised, self-administered, non-intravenous analgesia is available [23,24,39].

Patient-controlled analgesia with morphine has been evaluated in emergency settings, but its role depends on patient selection, monitoring capacity, staff resources, and departmental workflow [2,3,38].

Newer non-opioid strategies and emerging analgesic approaches may expand future emergency pain options, but implementation should be guided by evidence, safety, cost, access, and clinical practicality [3,4,8,35].

3.6. Trauma pain in the emergency department

Trauma-related pain is a major emergency department problem because injured patients often require simultaneous stabilization, diagnostic imaging, immobilization, procedural care, and analgesia [23,24,25,26].

Trauma pain management should begin early and should not be routinely postponed until the diagnostic work-up has been completed [2,23,24].

The reviewed trauma literature supports systematic pain assessment on admission and repeated evaluation during emergency department care [23,24,25,26].

In trauma patients, analgesic choice should account for hemodynamic status, injury severity, consciousness, respiratory risk, need for procedures, and route availability [2,23,24,26]. Studies from different emergency care systems show that trauma pain assessment and treatment may remain inconsistent even when pain is recognized as clinically important [25,26,27,28]. The global distribution of trauma pain studies suggests that oligoanalgesia is a broad emergency care problem that can occur in both high-resource and resource-limited settings [24,25,26,28].

3.7. Pediatric pain and other special populations

Children require age-appropriate pain assessment, developmentally appropriate communication, caregiver involvement, and careful selection of pharmacological and non-pharmacological strategies [1,2,32,33].

Pediatric pain is vulnerable to undertreatment because fear, developmental stage, communication barriers, procedural distress, and clinician uncertainty can complicate assessment [32,33,34].

Emergency care for pediatric pain should include both analgesia and distress reduction because anxiety and fear may intensify the child's pain experience and reduce cooperation [32,33,34].

Patients with sickle cell pain episodes require rapid and protocolized analgesic care because delayed treatment can prolong severe pain and worsen patient experience [22,34].

Older adults, pregnant and breastfeeding patients, patients with cancer pain, patients with palliative needs, and patients with complex comorbidities require individualized analgesic decisions because standard drug choices may be unsafe or insufficient [22,31,35].

End-of-life pain management in the emergency department requires attention to symptom relief, dignity, communication, goals of care, and avoidance of unnecessary suffering [22,31]. The reviewed literature supports an individualized approach in special populations because rapid analgesia must remain both timely and safe [2,22,31,32].

3.8. Organizational barriers and quality improvement

Emergency department pain management depends on organizational design as much as on pharmacological knowledge [17,18,19,20].

Barriers to effective analgesia include workload, crowding, lack of staff engagement, insufficient training, fragmented responsibility, poor documentation, medication access problems, and limited feedback on performance [16,18,19,20].

Lean-driven and pathway-based quality improvement studies indicate that pain management problems require system-level solutions rather than isolated educational reminders [19,20]. Structured pathways may improve reliability by clarifying when pain should be assessed, who can initiate treatment, which medications are available, and when reassessment should occur [17,18,20].

However, quality improvement interventions may have limited impact if they do not address clinical engagement, staff workload, protocol usability, and real-time barriers to medication administration [18,19,20].

The evidence supports combining staff education, triage protocols, reassessment standards, documentation prompts, audit, feedback, and leadership support to reduce oligoanalgesia [16,17,18,19].

4. Discussion

The reviewed literature supports the position that rapid analgesia should be treated as a core quality element of emergency medicine rather than as an optional comfort intervention [1,2,3,8].

The most consistent finding across guidelines, reviews, and quality improvement studies is that good emergency pain care requires a sequence of assessment, treatment, reassessment, and treatment adjustment [1,2,3,16].

This sequence is important because a single pain score at arrival cannot ensure that the patient has received effective or sustained analgesia [9,11,12,13].

The persistence of oligoanalgesia suggests that emergency departments often fail not because analgesic options are absent but because care processes are inconsistent or delayed [3,7,19,20].

Therefore, rapid analgesia should be understood as a process measure and a patient-centered outcome rather than simply as the administration of a drug [1,3,10,20].

A major implication of the reviewed evidence is that pain assessment must be structured but not mechanically reduced to a number [1,10,13,15].

Numeric or visual pain scales can support communication and prioritization, but they must be interpreted together with diagnosis, function, distress, risk, patient preference, and clinical trajectory [10,13,14,15].

This broader interpretation is especially important when high pain intensity does not automatically identify the safest treatment or when lower scores coexist with substantial functional impairment or distress [10,13,15].

The emerging focus on defining analgesic need reflects a shift from isolated pain scoring toward more comprehensive decision-making in emergency analgesia [14,15]. This shift is clinically useful because it allows clinicians to preserve the advantages of standardized assessment while avoiding overly rigid treatment decisions [1,13,15].

The evidence also indicates that oligoanalgesia is a system-level problem that requires system-level interventions [17,18,19,20].

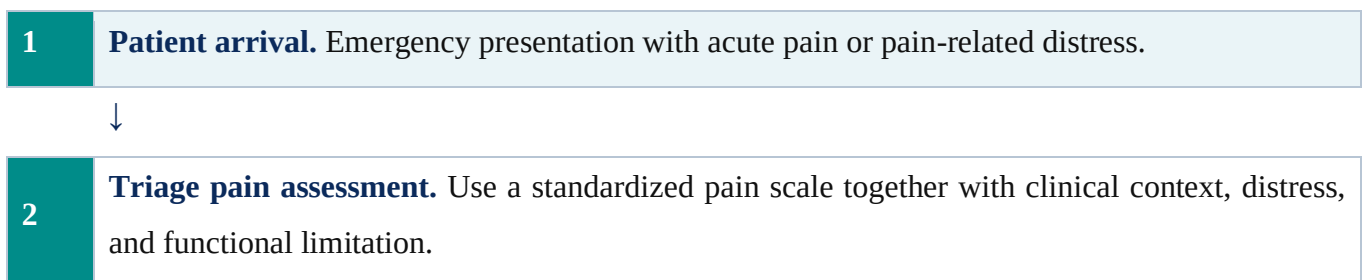
Educational interventions may improve knowledge and documentation, but they may not be sufficient if triage processes, prescribing authority, medication availability, and reassessment responsibilities remain unclear [16,17,19].

Nurse-initiated analgesia may reduce time to treatment in selected patients because it moves pain relief closer to the point of triage [17,18].

However, nurse-initiated protocols require careful medication selection, eligibility criteria, contraindication screening, staff training, and reassessment safeguards [2,17,18]. Emergency departments should therefore design analgesia pathways that are simple enough for real clinical conditions but flexible enough to accommodate patient complexity [17,18,20,22].

Figure 1. Rapid analgesia pathway in the emergency department

A practical stepwise approach for early, safe, and individualized pain management



↓

3 **Analgesic need stratification.** Classify pain severity and consider age, diagnosis, comorbidities, pregnancy, frailty, organ dysfunction, and respiratory risk.

↓

4 **Early analgesic decision.** Do not delay treatment unnecessarily when analgesia is clinically indicated.

↓

5 **Initial treatment.** Use non-pharmacological measures and appropriate pharmacological therapy, including multimodal strategies when indicated. Examples: positioning, splinting, acetaminophen, NSAIDs, opioids, ketamine, intranasal or inhaled analgesia, regional techniques.

↓

6 **Reassessment after intervention.** Recheck pain intensity, function, distress, sedation, respiratory status, and adverse effects.

↓

7 **Treatment adjustment.** Repeat dose, change drug, change route, add another modality, or escalate monitoring if needed.

↓

8 **Disposition and ongoing plan.** Prepare discharge, admission, or transfer with documentation, ongoing analgesia, safety advice, and follow-up plan.

Core principles	Common barriers
Timely • Multimodal • Individualized • Reassessed	Crowding • Workflow delays • Poor documentation • Limited reassessment

A practical pathway for rapid analgesia in the emergency department should integrate triage-based pain assessment, early analgesic decision-making, individualized treatment, reassessment, and treatment adjustment, as shown in Figure 1 [1–3,16–20,22,23].

Multimodal analgesia is strongly supported by the reviewed literature because emergency pain has diverse mechanisms and patient risk profiles [3,4,5,8].

This approach is clinically attractive because it allows clinicians to combine non-opioid medications, opioids, regional techniques, inhaled or intranasal routes, and non-pharmacological measures according to patient need [3,4,5,23]. Multimodal analgesia should not be interpreted as automatic polypharmacy because each component should have a clear indication, expected benefit, and acceptable safety profile [2,4,22,36].

Opioids remain necessary for selected patients with severe acute pain, but the literature supports avoiding unnecessary opioid exposure when effective non-opioid or opioid-sparing alternatives are appropriate [3,4,6,36].

Ketamine, methoxyflurane, intranasal analgesia, and patient-controlled analgesia may be useful in selected scenarios, but their implementation depends on evidence, local availability, monitoring capacity, and clinician familiarity [30,38,39,40].

Trauma pain illustrates the need to integrate analgesia with emergency diagnosis and stabilization [23,24,26].

Pain relief in trauma should not be delayed merely because imaging or procedural care is pending, because analgesia can coexist with repeated examination, monitoring, and diagnostic reassessment [2,23,24].

At the same time, trauma analgesia must account for hemodynamic instability, altered consciousness, respiratory risk, and the possibility of urgent procedures [2,23,26]. This balance shows that rapid analgesia is not opposed to diagnostic safety but should be integrated into safe emergency care [1,2,23].

Emergency departments that treat analgesia as part of the trauma pathway may reduce avoidable suffering while preserving clinical vigilance [20,23,24].

Special populations demonstrate why rapid analgesia must be individualized [22,31,32,34]. Children may need non-opioid strategies, caregiver involvement, distraction, topical or intranasal options, and procedure-specific distress reduction [32,33,34]. Older adults may be more vulnerable to adverse drug effects, drug interactions, renal impairment, delirium, and respiratory complications, which makes careful dosing and reassessment essential [4,22,31].

Patients with sickle cell pain episodes may require rapid protocolized analgesia because delayed treatment risks prolonging severe pain and worsening trust in emergency care [22,34]. Patients at the end of life require an approach focused on comfort, dignity, communication, and

alignment of emergency interventions with goals of care [31]. These examples show that rapid analgesia should mean timely individualized treatment rather than uniform treatment for all patients with the same pain score [1,2,22,32].

The ethical dimension of rapid analgesia is central to emergency medicine [3,5,8,9]. Untreated pain represents avoidable suffering and may negatively affect the patient's perception of safety, dignity, and trust in emergency care [8,9,10].

The historical concern that analgesia could mask diagnosis should not justify routine delays in pain treatment because modern emergency practice includes reassessment, imaging, laboratory testing, serial examination, and monitoring [1,2,23].

A more appropriate approach is to provide analgesia while continuing diagnostic evaluation and documenting clinical change after treatment [1,2,16,23].

This approach respects both patient comfort and diagnostic responsibility [1,2,3,8].

Future improvement should focus on practical, measurable, and sustainable emergency department interventions [17,18,19,20].

Useful quality indicators may include documentation of pain at triage, time to first analgesia, reassessment after treatment, rescue analgesia, adverse events, patient satisfaction, and equity across demographic groups [9,10,16,19].

Research should also examine how analgesia protocols perform under crowding, workload pressure, limited staffing, and resource constraints [18,19,20,28].

More evidence is needed on the best ways to combine triage-initiated analgesia, multimodal drug selection, non-pharmacological methods, and reassessment into routine emergency department workflow [17,18,20,23].

Overall, the reviewed evidence supports a model of emergency pain care that is early, structured, individualized, multimodal, and continuously reassessed [1,2,3,8].

5. Conclusion

Acute pain is a major clinical problem in emergency medicine and remains vulnerable to underassessment, delayed treatment, and insufficient reassessment [1,3,7,9].

Rapid analgesia should be recognized as a core element of emergency department care because pain relief is directly related to patient-centered quality and avoidance of unnecessary suffering [1,2,3,8].

Effective emergency pain management requires structured assessment, timely analgesic intervention, reassessment after treatment, and adjustment of therapy according to response and safety [1,2,3,16].

Oligoanalgesia should be understood as a workflow and system problem involving assessment, documentation, staffing, protocols, prescribing processes, medication access, and reassessment standards [17,18,19,20].

Multimodal analgesia provides a practical framework for combining non-opioid drugs, opioids, ketamine, inhaled or intranasal options, regional techniques, and non-pharmacological methods according to patient need [3,4,5,23].

Opioids remain appropriate for selected cases of severe acute pain, but opioid-sparing and non-opioid strategies should be used whenever they are effective and safe for the individual patient [3,4,6,36].

Trauma patients, children, older adults, pregnant patients, patients with sickle cell disease, patients with cancer pain, and patients receiving end-of-life care require individualized analgesic decisions rather than rigid treatment algorithms [22,23,31,32].

Emergency departments should implement staff education, triage-based protocols, nurse-initiated analgesia where appropriate, documentation prompts, reassessment standards, and quality improvement systems to reduce oligoanalgesia [16,17,18,19].

The central conclusion of this review is that rapid analgesia in the emergency department should be timely, safe, multimodal, individualized, and repeatedly reassessed [1,2,3,8].

Supplementary Materials

Not applicable.

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Conflicts of interest statement

The authors declare no conflict of interest.

Disclosure

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